



|            |                          |                  |                     |                     |                          |
|------------|--------------------------|------------------|---------------------|---------------------|--------------------------|
| Name       | Baby.OF MEENAKSHI SHARMA | Sample Received  | 02-11-2024 08:25 PM | Sample Collected At | Cure N Care Nursing Home |
| Age/Gender | 0Y-0M-5D / Male          | Sample Processed | 02-11-2024 08:25 PM | Referred By         | Cure N Care Nursing Home |
| UID        | HLD0211202441344         | Approved Date    | 02-11-2024 09:10 PM |                     |                          |

## IMMUNOLOGY REPORT

| Investigations      | Values | Unit   | Bio-logical Reference |
|---------------------|--------|--------|-----------------------|
| *ULTRASENSITIVE TSH | 4.341  | μIU/ml | 0.64 - 6.27           |

Method: C.L.I.A

## Remarks:

The biological reference range pertains to the age and sex of the Patient.

**Note:** As the reports are computer generated, it must be treated as an original.**Dr. Anuja Sharma**  
Consultant Pathologist  
Reg.No. 455**A Unit Of Hexamed Healthcare Services LLP****Corporate Office:** Flat No 203, 2nd Floor, Sagacity Heights, Plot No 27, Kavuri Hills Rd, CBI Colony, Madhapur, Telangana 500033**Jammu Address:** 25-270/6, Main Bypass Road, NH-1A, Channi Rama, Jammu, J & K 180015

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**BIO CHEMISTRY REPORT**

| Investigations   | Values       | Unit  | Bio-logical Reference |
|------------------|--------------|-------|-----------------------|
| *BILIRUBIN TOTAL | <b>11.13</b> | mg/dL | 0.3 - 1.1             |

Method: Vanadate oxidation

**Remarks:**

The biological reference range pertains to the age and sex of the Patient.

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|              |                     |                   |                      |
|--------------|---------------------|-------------------|----------------------|
| Barcode No   | 87943577            | Lab No            | 16952411030002       |
| Patient Name | B/OMEENAKSHI SHARMA | Reg Date          | 03/Nov/2024 02:11PM  |
| Age/Sex      | 5 DAYS(S)/Male      | Sample Coll. Date | 03/Nov/2024 01:13 PM |
| Referred By  | DR. JASPREET SINGH  | Sample Rec.Date   | 04/Nov/2024 07:43 AM |
|              |                     | Report Date       | 04/Nov/2024 11:50AM  |

## HAEMATOLOGY

| Test Name With Methodology            | Result | Unit   | Biological Ref.Interval |
|---------------------------------------|--------|--------|-------------------------|
| <b>G6PD Deficiency (Quantitative)</b> |        |        |                         |
| G6PD (Quantitative)                   | 12.3   | U/g Hb | >2.8                    |

EDTA Whole Blood, Spectrophotometry

### CUT-OFF VALUES

| Sex/G6PD Statu           | U/g Hb     |
|--------------------------|------------|
| Male/G6PD deficiency     | <2.8       |
| Male/G6PD normal         | ≥2.8       |
| Female/G6PD deficiency   | <2.8       |
| Female/G6PD intermediate | 2.8 – <7.4 |
| Female/G6PD normal       | ≥7.4       |

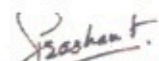
### Comment:

The goal of G6PD activity testing is to detect a G6PD deficiency and to determine its potential severity. It is primarily ordered on male children as they are the most frequently affected but may also be ordered on female children if the doctor suspects that a G6PD deficiency may be present. G6PD activity testing may be ordered on children who had persistent jaundice as a newborn that was not due to another identified cause. It may also be ordered on patients of any age who have had one or more episodes of hemolytic anemia,

If there is decreased G6PD activity, there is some degree of G6PD deficiency. In general, the lower the activity level, the more likely the patient is to experience symptoms when exposed to an oxidative stress. The results, however, cannot be used to predict how an affected patient will react in a given set of circumstances. The severity of symptoms will vary from patient to patient and from episode to episode.

If a male patient has normal G6PD activity levels, it is likely that they do not have a deficiency. However, if the test was performed during an episode of hemolytic anemia, it should be repeated a few weeks later when the RBC population has had time to replenish and mature.

Heterozygous females will have both G6PD-deficient and non-deficient cells. They will usually have normal or near normal G6PD activity levels, and few will experience any symptoms. A carrier may not be able to be detected through G6PD activity testing. In the rare homozygous female, G6PD activity will be significantly and detectably decreased.

  
 Dr. Prashant Goyal (DCP)  
 (Director & Chief Pathologist)  
 Reg. No. DMC-53016

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